ARC ANIMAL BITE CENTER

Clinic Management System

Final Business & System Requirements Document

Version 1.2 - Scope Baseline for Client Approval

Updated from client meetings, supplied ARC/DOH forms, and current system review on 26 October 2026.

Deployment Model	Centralized cloud-based web application (recommended; client approval required)
Branches	12
Current Development Scope	Clinic Management System including the defined Attendance & Payroll scope
Development Fee	PHP 120,000 (including the defined Attendance & Payroll scope; deployment/hosting/third-party recurring fees excluded)
Document Purpose	Define the approved scope, exclusions, remaining configuration items, and UAT requirements for the current implementation.

1. Purpose and Scope

This document defines the updated business and system requirements for a centralized Clinic Management System for ARC Animal Bite Center. It is intended to serve as the scope baseline for client approval, completion of remaining development, user acceptance testing, and deployment planning.

The system will digitize the clinic's patient admission, animal bite/RER workflow, treatment and vaccination follow-up, inventory, sales/payment recording, basic finance tracking, employee attendance/payroll, and DOH/clinic reporting across 12 branches.

2. Business Objectives

- Centralize clinic records across 12 branches while enforcing branch-level access.
- Reduce duplicate/manual encoding of patient, RER, treatment, and reporting data.
- Maintain longitudinal patient and treatment history for returning patients.
- Track vaccination schedules, succeeding doses, vaccinators, and follow-up status.
- Track vaccine/serum inventory per branch, including lot/batch and expiry information.
- Automatically derive operational and DOH reports from encoded patient and treatment transactions.
- Provide branch-level sales and a basic finance tracker without implementing a full accounting system.

3. Solution Scope and Architecture

The recommended solution is one centralized cloud-hosted web application and central database, accessible by authorized users from all 12 branches through a web browser. Separate independent databases/installations per branch are not part of the recommended design.

- Each operational record must carry a branch association.
- Branch users are restricted to their assigned branch and permitted modules.
- Superadmin/Admin users may access broader or all-branch information according to permissions.

- Production hosting, domain, SMS/messaging services, and other recurring infrastructure charges are separate from the development fee.
- Offline branch synchronization is not included.

4. Users, Roles and Access Control

ID	Requirement	Status / Notes
USR-01	Each authorized employee shall have an individual account.	Confirmed
USR-02	The system shall support role/permission-based access control.	Confirmed
USR-03	Branch-level users shall only see data for branches they are authorized to access.	Confirmed
USR-04	Superadmin/Admin shall have broader access based on the final permission matrix.	Confirmed
USR-05	The current system roles are Superadmin, Admin, Nurse, PhilHealth Officer and Secretary. Owner and Cashier are not separate application roles.	Current implemented role list; final operation permissions require UAT
USR-06	The final create/view/update/delete/export permission matrix shall be configurable/approved before UAT.	Configuration item

5. Patient and Admission Management

ID	Requirement	Status / Notes
PAT-01	Register new patients and retain a reusable patient master record.	Confirmed
PAT-02	Support returning patients and display previous clinic visits/treatments where authorized.	Confirmed
PAT-03	Search patients by name, admission/registry number, and contact number.	Confirmed
PAT-04	Generate a unique admission/registry number automatically according to the final numbering convention.	Confirmed; current format ARC{branch code}-YY-0001; verify with the clinic
PAT-05	Capture patient demographic/contact data required by the ARC admission/RER form, including name, birthday/age, gender, cellphone, address, guardian where applicable, and relevant patient flags such as Senior/PWD/PhilHealth/Vetmed where required by the form.	Form-derived
PAT-06	Associate each admission/encounter with the servicing branch.	Confirmed
PAT-07	Avoid duplicate encoding by reusing patient data in RER/treatment/reporting workflows.	Confirmed objective

6. Animal Bite / RER Management

The digital RER shall follow the supplied ARC Animal Bite Center form and capture structured data sufficient for treatment follow-up and reporting.

ID	Requirement	Status / Notes
RER-01	Capture biting animal (Dog, Cat, Other) and type of exposure (Bite/Non-Bite).	Form-derived
RER-02	Capture pet/owned/stray information, vaccination status of animal where recorded, owner information where applicable, date/time of bite, place of bite, and wound washing/tandok indicators as represented on the form.	Form-derived

ID	Requirement	Status / Notes
RER-03	Capture wound type such as abrasion, lacerated, bruised, healed, plus affected body site.	Form-derived
RER-04	Capture Category I, II, III and booster/pre-exposure indicators as applicable.	Confirmed/form-derived
RER-05	Capture vaccine type/product/brand and route/dose information using controlled selections where practical.	Confirmed
RER-06	Capture relevant medical history/vitals represented on the supplied form (e.g., BP, weight, temperature and listed history indicators).	Form-derived
RER-07	Support consent/signature information and doctor/vaccinator signature data where required.	Form-derived
RER-08	Support final record status/remarks including Complete, Incomplete, and Transferred.	Form-derived
RER-09	Provide printable/exportable patient/RER records.	Confirmed; exact layouts during design

7. Treatment, Vaccination and Follow-up

ID	Requirement	Status / Notes
TRT-01	Record each administered anti-rabies vaccine/treatment as a transaction linked to patient, encounter, branch and vaccinator.	Confirmed
TRT-02	Support treatment schedule entries corresponding to clinic-approved protocols, including the supplied form's D0, D3, D7, D14 and D21/28/D28-type follow-up fields as applicable.	Protocol must be clinic-approved
TRT-03	Record route/dose and vaccine brand/product for each administered dose.	Confirmed/form-derived
TRT-04	Record status of biting animal during follow-up (e.g., Alive, Lost/Stray, Dead) where required.	Form-derived
TRT-05	Record vaccinator for each dose and support stored e-signature/signature reference where approved.	Confirmed/discussed
TRT-06	Support Tetanus Toxoid information and ERIG/RIG information represented on the supplied ARC form.	Form-derived
TRT-07	Clinic personnel determine medical assessment/category; the system records the selected assessment and does not independently diagnose patients.	Business rule
TRT-08	Maintain complete treatment history for returning patients.	Confirmed
TRT-09	The current schedule is ID D0/D3/D7/D28, with D14 added when the animal is reported dead or lost within the 14-day observation window; IM D0/D3/D7/D14/D28; booster D0/D3; and PrEP D0/D7/D21.	Current implementation; clinic approval and UAT required
TRT-10	More than seven days past the next due date marks the case Incomplete. Staff may still record a late dose; remaining due dates recalculate from its actual administration date.	Current implementation; clinic approval and UAT required

8. Progress Notes

The supplied Progress Notes form uses repeated SOAP-style entries. The system shall support multiple progress notes per patient/encounter.

ID	Requirement	Status / Notes
PRG-01	Allow multiple dated progress-note entries linked to the patient/encounter.	Form-derived
PRG-02	Support Subjective information such as fever, rashes, numbness at wound site, headaches, other symptoms, and biting-animal status.	Form-derived
PRG-03	Support Objective information such as body temperature, BP/HR, general weakness, hydrophobia, aerophobia, photophobia, rashes and other findings.	Form-derived
PRG-04	Support Assessment entries such as status post animal bite, healed wound secondary to animal bite, and other assessment text/selections.	Form-derived
PRG-05	Support Plan entries such as vaccine day administered, ERIG, next return day, continued observation of biting animal, and other instructions.	Form-derived
PRG-06	Record the responsible staff name/signature for each progress note. Patient and visit registry are linked automatically; same-day recorded BP, temperature and animal status may prefill for clinician review. Current symptoms, assessment and plan require clinician entry.	Form-derived

9. Scheduling and Reminders

ID	Requirement	Status / Notes
SCH-01	Calculate/display succeeding treatment dates from the clinic-approved schedule.	Confirmed
SCH-02	Provide staff visibility of patients with upcoming or due doses.	Confirmed
SCH-03	Support patient reminders for upcoming doses.	Confirmed concept
SCH-04	External reminder channel (SMS/email/other) and any third-party provider shall be configured separately; provider usage charges are excluded from the development fee.	Pending channel selection

10. Vaccine and Serum Inventory

ID	Requirement	Status / Notes
INV-01	Maintain inventory independently per branch.	Confirmed
INV-02	Maintain vaccine/serum item master information using vial/piece/bottle as applicable, with approved consumption rules per item.	Confirmed; item-specific rules
INV-03	Record Stock In/receiving transactions.	Confirmed
INV-04	Record Stock Out/usage transactions by branch, date and shift (Morning Shift / Night Shift) and calculate current balance.	Confirmed
INV-05	Track batch/lot number and expiration date.	Confirmed
INV-06	Provide low-stock and near-expiry/expired visibility or alerts.	Confirmed/requested
INV-07	Treatment-related inventory usage shall update stock according to the clinic-approved consumption rule for the selected item; a vial is deducted when fully consumed.	Confirmed business rule
INV-08	Maintain inventory transaction history/remarks for traceability.	Scope baseline
INV-09	Supplier management is not included.	Explicit exclusion
INV-10	Support Morning Shift (MS) and Night Shift (NS) inventory monitoring, with previous ending stock carried forward as the next applicable beginning stock.	Confirmed
INV-11	For Tetanus Toxoid, one patient treatment consumes one vial.	Confirmed
INV-12	Bioshoot, Speeda and Abhayrab each use five injection-use units/sites per vial. Usage accumulates by actual injection and one vial is deducted when fully consumed.	Confirmed
INV-13	Allow stock adjustments for damaged, expired, missing, correction or other approved reasons, with reason, user and audit trace.	Required for traceability
INV-14	For Flu vaccine, one patient treatment consumes one vial.	Required for UAT; automatic treatment deduction is not yet implemented
INV-15	For Pneumococcal vaccine, one patient treatment consumes one vial.	Required for UAT; automatic treatment deduction is not yet implemented
INV-16	ChiroRab and VaxiRab N (PCEC) each use ten injection-use units/sites per vial. The open-vial counter displays uses out of ten; one vial is deducted when fully consumed.	Current consumption rule; verify against approved clinic protocol
INV-17	ERIG (serum) stock is recorded through manual Stock Out according to actual vials used; it is not deducted automatically from the treatment entry.	Current workflow; verify actual usage with clinic staff

11. PhilHealth / Walk-in, Services, Payments and Sales

ID	Requirement	Status / Notes
SAL-01	Record applicable PhilHealth/non-PhilHealth or walk-in classification required by clinic workflow/reporting.	Confirmed
SAL-02	Maintain clinic service/treatment pricing needed for payment recording.	Confirmed
SAL-03	Record payment amount, patient/encounter, branch and transaction date.	Confirmed
SAL-04	Compute sales from recorded transactions, including daily, monthly, annual and date-range totals, with branch-level views.	Confirmed
SAL-05	Official receipt/invoice generation is excluded; the client will use a separate receipt device.	Explicit exclusion
SAL-06	Support cross-branch patient referral while preserving one longitudinal patient/treatment history across authorized branches.	Confirmed workflow
SAL-07	Track the referring branch, receiving branch, referral date/status and reason, including referral to an accredited branch for succeeding treatment where applicable.	Confirmed workflow
SAL-08	When a prior cash payment becomes refundable because of the approved PhilHealth/cross-branch workflow, preserve the original payment and create a separate refund transaction instead of deleting the payment.	Confirmed workflow
SAL-09	The refund is claimed/processed at the original branch where the patient paid (e.g., Cuyapo in the confirmed scenario), subject to authorized approval and audit trail.	Confirmed
SAL-10	Refund records shall include amount, reason, source payment/reference, status, approving/processing user, branch and refund date.	Required
SAL-11	Branch referral begins at a non-accredited branch and goes to a different PhilHealth-accredited branch. Acceptance creates a linked receiving registry and PhilHealth classification; an eligible earlier cash payment stays in history while a separate refund request is opened at the paying branch.	Current workflow; validate referral and refund cases in UAT

12. Basic Finance Tracker

Finance scope is intentionally limited to a basic operational tracker and shall not be treated as a full accounting platform.

ID	Requirement	Status / Notes
FIN-01	Record branch-level cash-in/income entries not already represented by sales where needed.	Basic scope
FIN-02	Record expenses/cash-out with date, category/description, amount, remarks/reference and branch.	Basic scope
FIN-03	Show total income, total expenses and simple balance/summary for a selected period and branch.	Basic scope
FIN-04	Provide daily/monthly/date-range finance summaries.	Basic scope
FIN-05	Full accounting, general ledger, taxation/BIR computation, accounts payable/receivable and formal financial statements are excluded.	Explicit exclusion

13. Employee Attendance and Payroll

The Payroll scope is limited to employee attendance, daily-rate payroll computation, approved additions/deductions, statutory contribution computation, payslips and payroll reporting. It is not a full HRIS.

ID	Requirement	Status / Notes
PAY-01	Maintain employee records including assigned branch, position/role, employment status, daily rate, shift and payroll eligibility.	Confirmed
PAY-02	Provide an individual system account for each employee who uses the system; employee login and attendance are separate actions. Employees explicitly Time In and Time Out.	Current implemented workflow
PAY-03	Support Morning Shift and Night Shift assignment. Night duty currently applies to Guimba (Main Branch, Daily 7 AM–11 PM) and Cabanatuan (Daily 7 AM–11 PM); applicability shall be configuration-driven rather than hard-coded.	Confirmed/current configuration
PAY-04	Support a Time Out attendance action/workflow and attendance history. Employee attendance timestamps shall not be directly editable by ordinary employees.	Required; final UI workflow during design
PAY-05	Allow authorized Admin/HR users to correct attendance only with a required reason and audit trail. The existing Bundy clock remains an independent/secondary verification source; direct device integration is excluded unless separately added.	Confirmed scope
PAY-06	Payroll is semi-monthly. Payroll release dates are the 15th and 30th; cutoff dates are the 10th and 25th.	Confirmed
PAY-07	Employees are paid on a daily-rate, no-work-no-pay basis. Basic pay is computed from the employee daily rate and actual approved duty days within the payroll period.	Confirmed
PAY-08	Late and undertime deductions shall use exact minutes: Daily Rate ÷ Approved Working Hours per Day ÷ 60 × Late/Undertime Minutes.	Confirmed
PAY-09	Overtime is not automatically granted. OT is payable only when required and approved by an authorized management approver; approved OT hours and approval details shall be recorded.	Confirmed
PAY-10	Approved overtime pay shall be based on the employee hourly salary rate and approved OT hours, subject to the final approved payroll rule/configuration.	Confirmed baseline
PAY-11	Night differential applies only to actual night-duty hours and is computed at 10% of the employee basic hourly rate per applicable night hour.	Confirmed
PAY-12	Officer allowance is PHP 500 per cutoff (PHP 1,000 per month). Eligibility shall be configurable per employee rather than hard-coded.	Confirmed
PAY-13	Support holiday classification and payroll computation: Regular Holiday = +100%; Special Holiday = +30%, based on actual eligible duty and the approved payroll formula.	Confirmed
PAY-14	Automatically compute SSS, PhilHealth and Pag-IBIG deductions using configurable contribution tables/rules and effective dates.	Confirmed requirement
PAY-15	Allow authorized per-employee override/custom amounts for SSS, PhilHealth and Pag-IBIG where approved, with reason, effective period, user and audit trail.	Confirmed special-case requirement
PAY-16	Generate payroll records showing basic pay, duty days, late/undertime, approved OT, night differential, holiday pay, allowance, mandatory deductions, other approved adjustments and net pay.	Required
PAY-17	Generate printable/downloadable employee payslips for each payroll period.	Confirmed
PAY-18	Provide payroll history and reports by employee, branch, cutoff/pay period and consolidated view for authorized Owner/Admin/HR users.	Required

ID	Requirement	Status / Notes
PAY-19	No paid leave/leave-credit computation is included in the current scope; current rule is no work, no pay.	Confirmed

14. DOH / Clinic Reporting

Reporting shall be derived from structured patient, exposure and treatment transactions. Users should not need to manually re-encode the same case into a separate report when the required data already exists in the system.

14.1 Rabies Exposure Registry

ID	Requirement	Status / Notes
REP-01	Generate a Rabies Exposure Registry based on the supplied DOH National Rabies Prevention and Control Program registry format.	Form-derived
REP-02	Include registration/patient data, exposure history, washing of bite, RIG date, PEP route, vaccine dates (D0/D3/D7/D14/D28 as applicable), brand, outcome, pre-exposure/booster, animal status and remarks as represented by the supplied registry.	Form-derived
REP-03	Support print/export of the registry, with print and XLSX export. PDF download remains an open requirement where applicable.	Print and XLSX implemented; PDF download pending

14.2 Animal Bite and Rabies Aggregate Report

ID	Requirement	Status / Notes
REP-04	Generate the DOH Animal Bite and Rabies Report for the selected reporting period and branch/location using the supplied form as reference.	Form-derived
REP-05	Automatically compute Sex totals (Male/Female/Total) and Age group totals (<15 and >15 as represented on the supplied form).	Form-derived
REP-06	Automatically compute Category I, Category II and Category III figures from encoded cases.	Form-derived
REP-07	Support PEP-related counts/coverage including primary/booster and CCEEV-related fields represented on the form.	Form-derived; exact formulas must match approved DOH template
REP-08	Support RIG/ERIG/HRIG-related counts where represented on the report.	Form-derived
REP-09	Compute biting-animal totals by type (Dog, Cat, Others) and status (Pet/Domestic, Stray/Free-roaming, Unknown) where required.	Form-derived
REP-10	Support human rabies case/incidence fields represented on the supplied report when data is available.	Form-derived
REP-11	Reports shall support monthly, quarterly and annual/date-range views as applicable to the clinic's reporting workflow.	Confirmed/discussed
REP-12	Report formulas, code definitions and medical/reporting classifications shall follow the final clinic/DOH-approved template; ambiguous handwritten annotations are not treated as rules until confirmed.	Control rule

15. Dashboard

The dashboard shall summarize already-captured operational data according to user permissions. Exact widgets will be finalized during UI design.

- Patient/admission activity.
- Upcoming/due vaccination schedules.
- Treatment completion/follow-up indicators.
- Inventory balances, low stock and expiry indicators.
- Sales and basic finance summaries.
- Branch-level or all-branch summaries according to permission.

16. Search, Filtering, Printing and Export

ID	Requirement	Status / Notes
GEN-01	Provide practical search/filtering within major modules.	Required
GEN-02	Support branch and date/date-range filtering where relevant.	Required
GEN-03	Support printable output for applicable forms/reports.	Required
GEN-04	Support XLSX export and printing for applicable reports/registries. PDF download remains required where approved and must pass UAT before it is represented as delivered.	XLSX and print implemented; PDF download pending

17. Data Integrity, Audit and Security Requirements

ID	Requirement	Status / Notes
SEC-01	Use authenticated access and enforce server-side authorization/branch restrictions.	Required
SEC-02	Protect application traffic using HTTPS in production.	Required for cloud production
SEC-03	Database access shall not be directly exposed to ordinary internet users.	Required
SEC-04	Maintain audit/trace information for material record changes where practical, especially patient/treatment/inventory/finance transactions.	System requirement
SEC-05	Implement production backup procedures for database and uploaded files.	Required
SEC-06	Sensitive patient information shall only be visible to authorized users.	Required
SEC-07	Production security, backup retention and recovery procedures shall be finalized during deployment design.	Deployment configuration

18. Non-Functional Requirements

- Web-based and usable through current desktop/laptop browsers at clinic branches.
- Responsive enough for normal clinic operations and common screen sizes.
- Centralized data consistency across branches.
- Maintainable modular design so forms/report templates can be adjusted when approved requirements change.
- Reasonable validation to prevent incomplete/invalid required fields and duplicate/contradictory transactions.

- Production environment shall support routine backup and recovery.

19. Explicit Exclusions

- Full HRIS functions such as recruitment, performance management, employee loans, and paid leave/leave-credit management.

Direct Bundy clock/biometric hardware integration; the existing Bundy clock is retained as an independent verification source unless a separate integration is later approved.

- Official receipt/invoice generation.
- Full accounting/general ledger/tax/BIR system.
- Supplier management.
- Integration with unrelated clinics' databases.
- Automatic medical diagnosis or autonomous clinical decision-making.
- Offline branch synchronization.
- Third-party SMS/email/messaging usage fees.
- Cloud/VPS recurring fees, domain fees and other third-party recurring infrastructure charges.
- Features not described in this approved scope unless added through a change request.

20. Deployment and Infrastructure

Recommended production deployment: one centralized cloud-hosted application/database for all 12 branches. The client should preferably own the production cloud/domain accounts and billing, while the developer is granted the access required for setup and maintenance.

- Initial production deployment/configuration may be included as implementation work, subject to the final commercial agreement.
- Actual cloud/VPS, domain, backup storage, messaging and similar recurring provider costs are paid separately by the client.
- All branches require reliable internet access to use a centralized cloud system.
- Offline operation is outside the current scope.

21. Client Inputs / Configuration Items Before Final UAT

- Final user-role permission matrix.
- Official vaccine, serum/RIG, brand/product and route/dose master lists.
- Clinic-approved vaccination/treatment schedule rules.
- Final admission/registry numbering convention.
- Final item master list, beginning balances, batch/expiry data, low-stock thresholds and any additional item-specific consumption rules not already confirmed.
- Final reminder channel/provider if external patient messaging will be enabled.
- Confirm the RER consent and signature method, including whether electronic signature capture or Save Draft/Finalize is required. These details are open design decisions until the client approves them.
- Final current DOH/clinic reporting templates if they change from the supplied samples.
- Branch master list, authorized users, employee master list, daily rates, shift assignments and payroll roles/approvers.
- Finance categories to be used by the basic finance tracker.

Current/effective SSS, PhilHealth and Pag-IBIG contribution tables/rules to be configured and maintained by authorized users.

Officer-allowance eligible employees (initial client-provided list: Lyka Tenorio, Kim Diaz, Amery Maligaya, Michelle Clor, and Gelai Aban).

Employees assigned to night duty and the approved daily working hours used for hourly/late/undertime computations.

- Production cloud/domain account details when deployment begins.

22. Acceptance Criteria

- Authorized users can log in and access only permitted modules/branches.
- A new or returning patient can be registered/searched and an encounter/admission can be created.
- RER/exposure information and multiple treatment/follow-up records can be encoded and retrieved.
- Vaccination schedules and due follow-ups are visible and treatment history is retained.
- Inventory stock transactions and treatment-related deductions operate according to approved rules.

Cross-branch referrals and approved refunds retain the original payment history and produce traceable refund records at the originating branch.

Employee attendance, duty days, late/undertime, approved OT, night differential, holiday pay, allowance and mandatory deductions produce the expected payroll result for approved test cases.

Authorized users can generate payroll history and payslips for the selected cutoff/pay period.

- Payments/sales and basic finance entries produce correct branch/date-range summaries.
- Rabies Exposure Registry and agreed DOH aggregate reports are generated from encoded records with approved computations.
- Applicable forms/reports can be printed/exported.
- Production backup and access-control checks pass before go-live.
- Client/UAT representatives confirm that the agreed workflows and reports conform to the approved scope.

23. Commercial Scope Baseline

Item	Current Baseline
Development Fee	PHP 120,000 - agreed development scope including the defined Attendance & Payroll module
Payroll Scope	Included only as defined in Section 13; full HRIS and direct attendance-device integration remain excluded
Cloud/Hosting/Domain	Excluded from development fee; recurring client expense
Third-party Messaging	Excluded from development fee; usage/provider cost paid separately
Scope Changes	Material additions after approval should be evaluated and quoted separately
Payment Terms	Milestone-based payment schedule totaling PHP 120,000. The first two milestones total 70% (PHP 84,000) and are due together after the final presentation and system demonstration.
Downpayment / Project Start	40% - PHP 48,000, due after the final presentation and system demonstration, together with the Core Development Milestone, as agreed by the parties.
Core Development Milestone	30% - PHP 36,000, due after the final presentation and system demonstration of the agreed major/core modules, together with the Downpayment / Project Start milestone.
UAT Milestone	20% - PHP 24,000, due upon commencement/completion of User Acceptance Testing (UAT), subject to the agreed UAT process.
Final Deployment / Turnover	10% - PHP 12,000, due upon final deployment and project turnover.

24. Approval / Scope Freeze

Once this Version 1.2 document and the remaining configuration items are confirmed, it becomes the approved scope baseline for the current system and remaining development, testing and deployment work. Material features or workflows requested after scope approval should be handled through a documented change request and may affect price/timeline.

Approval of this document confirms the agreed business and system scope and the unchanged commercial amounts in Section 23. It is not confirmation that implementation, clinical validation, user acceptance testing, security checks or production deployment are complete.

Before go-live, the parties will verify treatment schedules, inventory and payroll calculations, role and branch permissions, DOH report totals, production backup and recovery, and remaining PDF/settings work. Client-provided master data and current contribution rules in Section 21 still require confirmation. Payments already made, if any, are tracked separately from this scope document.

_____ _____

Client representative name and signature Date

_____ _____

Developer or project representative name and signature Date